

COOK COUNTY PEDIATRICS

Primary Care

2875 W. 19th Street, Chicago, Illinois 60623

Phone: (312) 555-2200 | Fax: (312) 555-2209

OFFICE VISIT NOTE

Patient: Marcus D. Whitfield **MRN:** PED-2025-04471
DOB: 03/03/2016 **Age:** 8 years, 10 months **Sex:** Male
Date of Visit: 01/15/2025 **Visit Type:** Follow-up, chronic care
Provider: Anita Sharma, MD — Pediatrics
Accompanied By: Foster parent (prior placement)

SUBJECTIVE

Marcus is seen today for chronic care follow-up. Caregiver reports ongoing behavioral difficulties, poor appetite, and intermittent abdominal pain. He has been compliant with medications per caregiver report, though she is uncertain about all medication names and doses. He continues to attend school with IEP accommodations. Caregiver reports he snores loudly at night and sometimes seems to stop breathing briefly during sleep.

ACTIVE MEDICATION LIST

- Risperidone 1.5 mg QHS — Dr. Osei (Psychiatry)
- Methylphenidate ER 27 mg QAM — Dr. Liang (Dev Peds)
- Clonidine 0.1 mg QHS — Dr. Osei
- Sertraline 50 mg QHS — Dr. Osei
- Fluoxetine 10 mg QAM — started per psychiatry (per prior records)
- Budesonide/Formoterol 80/4.5 mcg 2 puffs BID
- Albuterol MDI 2 puffs PRN
- Montelukast 5 mg QHS
- PEG 3350 17 g daily
- Melatonin 3 mg QHS (OTC)

OBJECTIVE

Vital Signs: Temp 98.4 F, HR 88, RR 18, BP 92/58, SpO2 97%. Weight 24.6 kg (5th %ile), Height 129 cm (15th %ile). BMI 14.8 (<5th %ile).

General: Thin, quiet male. HEENT: Tonsils 3+, no erythema. Lungs: Clear bilaterally, no wheeze at rest. Abdomen: Soft, mild periumbilical tenderness on deep palpation. Skin: Multiple cafe au lait macules. Neuro: Limited eye contact, stereotypic hand movements (baseline).

ASSESSMENT AND PLAN

1. Asthma, severe persistent — currently stable on controller therapy. Three prior intubations. Continue Symbicort and albuterol PRN. Asthma action plan reviewed with caregiver.
2. Growth failure — ongoing. BMI <5th percentile. Multifactorial etiology suspected: stimulant effect on appetite, possible GI malabsorption, psychosocial factors. GI referral placed 3 months ago; appointment pending.
3. Behavioral concerns — defer to psychiatry and developmental pediatrics for medication management.
4. Sleep concerns — caregiver reports loud snoring and witnessed apneas. Given tonsils 3+ and history of neurodevelopmental disorders, polysomnography referral placed to Sleep Medicine for evaluation of suspected obstructive sleep apnea. Referral faxed today.
5. Pica — ongoing. Blood lead level 12 mcg/dL at last check (07/2025 pending lab). Environmental assessment recommended; discussed with DCFS caseworker.
6. Elevated blood lead — monitor serially.

Follow-up in 3 months or sooner if concerns.

Anita Sharma, MD

Pediatrics

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Electronically signed: 01/15/2025 at 16:45 CST
Dictated and finalized on 01/15/2025